Objective -

- Causes
- C/F
- INV.
- Rx
- A bit specifically on Variceal Rx

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UGIB

- Bleeding from UGI tract
- UGIT - Above Ligament of Treitz.
- Patients generally die from decompensation rather than exsanguination

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CAUSES

① **Peptic Ulcer** – M/c cause
- Data: • ½ of causes of hospitalization by UGIB
- • 1/5th of patients will have active bleeding or have visible vessel associated to the ulcer on endoscopy

② **Mallory Weiss Tears** – Classic h/o vomiting, retching, preceeding haematemesis
- Generally stops spontaneously in 80-90% cases, recurs in 0-10%
- ENDOSCOPIC Rx ONLY IN ACTIVE BLEEDING

③ **Esophageal Varices** – Poorest of all ~~common~~ causes
- Mainly cirrhotics (Portal HTN ↓ves)
- also Gastric varices

④ **Erosive gastropathy** –
- causes
  - Stress (CURLING) – Trauma, Sx, Burns > 1/3rd of B.S.A.
  - IC disease (CUSHING)
  - NSAID (MOST COMMON)
  - Alcoholics
  - H. Pylori (uncommon)

Mild UGIB due to mucosal irritation and erosion

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⑤ Neoplasms

⑥ GAVE - Gastric Antral vascular ectasias - WATER MELON STOMACH

⑦ Osler Weber Rendu Disease

⑧ Dieulafoy's lesion - Aberrant mucosal artery

⑨ Prolapse Gastropathy - Especially A/with repeated vomiting episodes in alcoholics

⑩ Aorto enteric fistulas

⑪ Haemobilia / Haemosuccus pancreaticus -

Bleeding from Bile duct / Pancreatic duct

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C/E

- Specific Symptoms:
  - Haematemesis
    - Fresh blood
    - Altered blood (coffee ground material)
  - Melaena (Black Tarry stool) — 14hrs, 3-5days,
  - Occult Blood???
  - Haematochezia??? (A/with Haemodynamic Instability & dropping Hb)

- Nonspecific Symptoms -
  - Signs of Haemodynamic Instability / Haemorrhagic Shock
    - Pulse, B.P, Shock Index (Pulse / S.BP), Pallor, Postural, JVP
      - Hypotension
      - Tachycardia

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Mx

* ESTABLISH IV access (large bore cannula)

* Initial Clinical assessment
  - Look for
    1. Specific clinical Signs & Symptoms
    2. Non specific signs & Symptoms
    3. Causes – Cirrhosis ESP., Drugs, Stress related,
    4. comorbidities – CLD, COPD, CKD, DM, HTN etc.
    5. Compute along the lines of GLASGOW BLATCHFORD Scale

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TABLE 44-1 Glasgow-Blatchford Score

ADMISSION MARKER	SCORE
Blood urea nitrogen (mg/dL)	
18.2 to <22.4	2
22.4 to <28.0	3
28.0 to <70.0	4
≥70.0	6
Hemoglobin (g/dL)	
12.0 to <13.0 (men); 10.0 to <12.0 (women)	1
10.0 to <12.0 (men)	3
<10.0	6
Systolic blood pressure (mmHg)	
100–109	1
90–99	2
<90	3
Heart rate (beats per minute)	
≥100	1
Other markers	
Melena	1
Syncope	2
Hepatic disease	2
Cardiac failure	2

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* **Basic Inv.**

→ CBC (The funda of Hb - 72 hrs)

→ U&E + BUN + S. creat (BUN↑d by (IV) S creat is what is assumed in UGIB with no comorb.)

→ LFT

→ PT (Bleeding profile)

→ Cross matching (≥ 2 units of blood may be req'd SO BE READY!!)

* **Resuscitation**

- ABC + O_2 (in shock) + Endotracheal intubation (if reqd-)
- IV fluid? - Crystalloids generally
- Blood? - Harrisson recommends at <7 g/dl (8? 9?)

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* **Definitive Rx** : Differs from causes to causes

└ (I) Pharmacological:

> PPI infusion - 2 types

a. Intensive - (IV / Oral) 80 mg bolus → 8 mg/h for 3 d.
→ Intermittent high dose (40-80 mg) for 3 d.

b. (N) - OD PPI

↓

4-14 d - BD PPI

↓

OD PPI

> Vasoactives in Varices -

Octreotide - 50 µg IV bolus followed by 50 µg/h infusion 2-5 d.

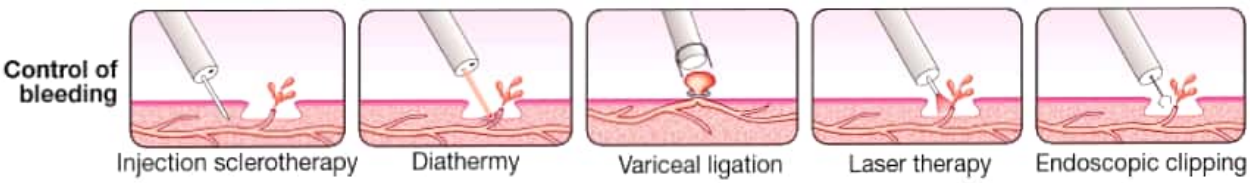
Terlipressin - 2 mg IV 4 times daily till bleeding stops → 1 mg 4 times daily for next 72 hrs

(SPECIFIC Rx PROCEDURES OF VARICES TO BE DISCUSSED LATER)

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(2) ENDOSCOPY :

- Timing - Within 24hrs (FOR VARICES - 12hrs)
- 2 purpose - dx + tc
  - Therapeutic : Sclerosant, diathermy, ligation, Laser Rx, clipping, Glue, Coagulant
    CTC 325 - haemostasis



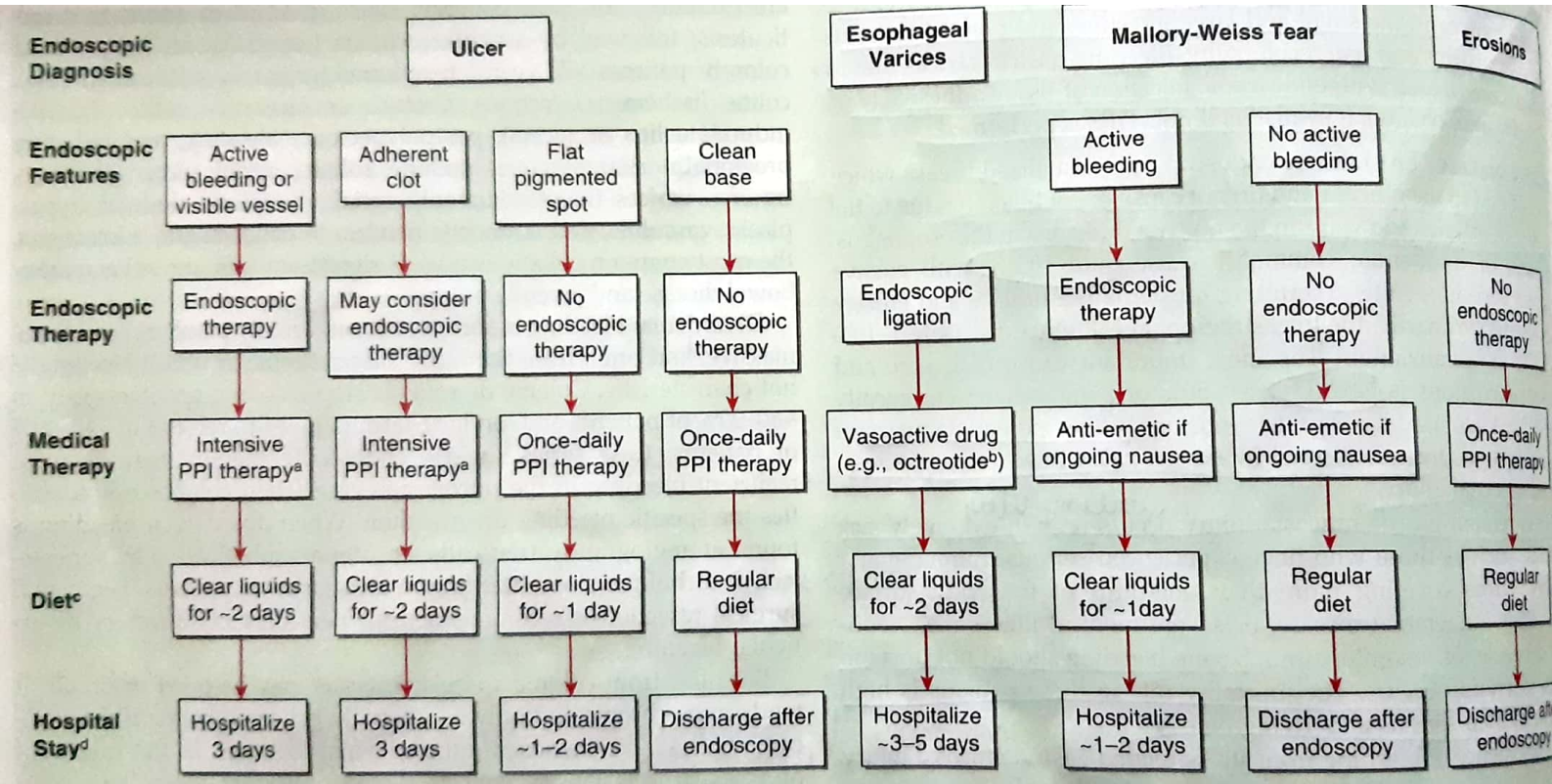
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③ [redacted] ● PLAN B procedures

when endoscopy fails :

- Balloon Tamponade (Sengstaken Blackemore, Linton, Minnesota) etc.)
- ANGIOGRAPHIC EMBOLISATION (esp in Surgical risk patients)
- SURGERY (depends on the site and type of disease)

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[a]Intravenous bolus (80 mg) followed by infusion (8 mg/h) for 3 days; or oral or intravenous bolus (e.g., 80 mg) followed by intermittent high doses (e.g., 40–80 bid or 40 mg tid) for 3 days. Then twice-daily PPI on days 4–14 followed by once-daily PPI.
[b]Intravenous 50 µg bolus followed by 50 µg/h infusion for 2–5 days.
[c]Diet after endoscopy, assuming no nausea or vomiting.
[d]Duration after endoscopy assuming patient stable without further bleeding or concurrent medical conditions requiring hospitalization; PPI, proton pump inhibi

FIGURE 44-1 Suggested algorithm for patients with acute upper gastrointestinal bleeding based on endoscopic findings.

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* Eradication: Prevention 2° Prophylaxis –

→ Ulcers: – NSAIDS, Acid, H pylori

NSAIDS → Stop, Cro for COX-2 if necessary

Acid → PPI for life if its the main cause

H pylori → Eradication Rx

→ Varices – BB blockers + Regular Endoscopic Band Ligation till veins obliterated (MORE DISCUSSED LATER)

→ Erosive – PPI

What about Aspirin? –

* If it was given as 1° prevention, that stop indefinitely
* If it was given as 2° prevention (established CVD / stroke) then restart ASAP after bleeding ends.

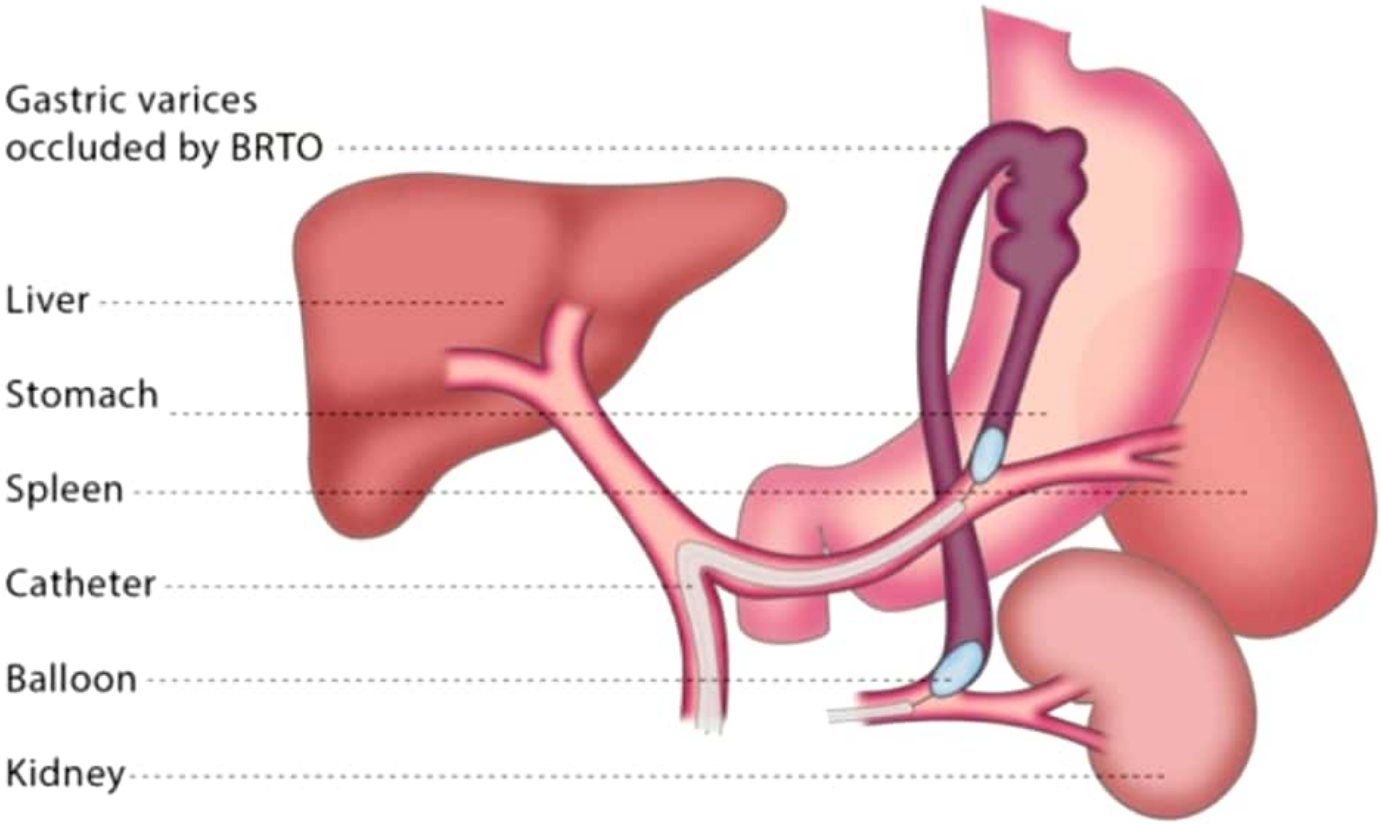
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VARICES – Definitive Rx

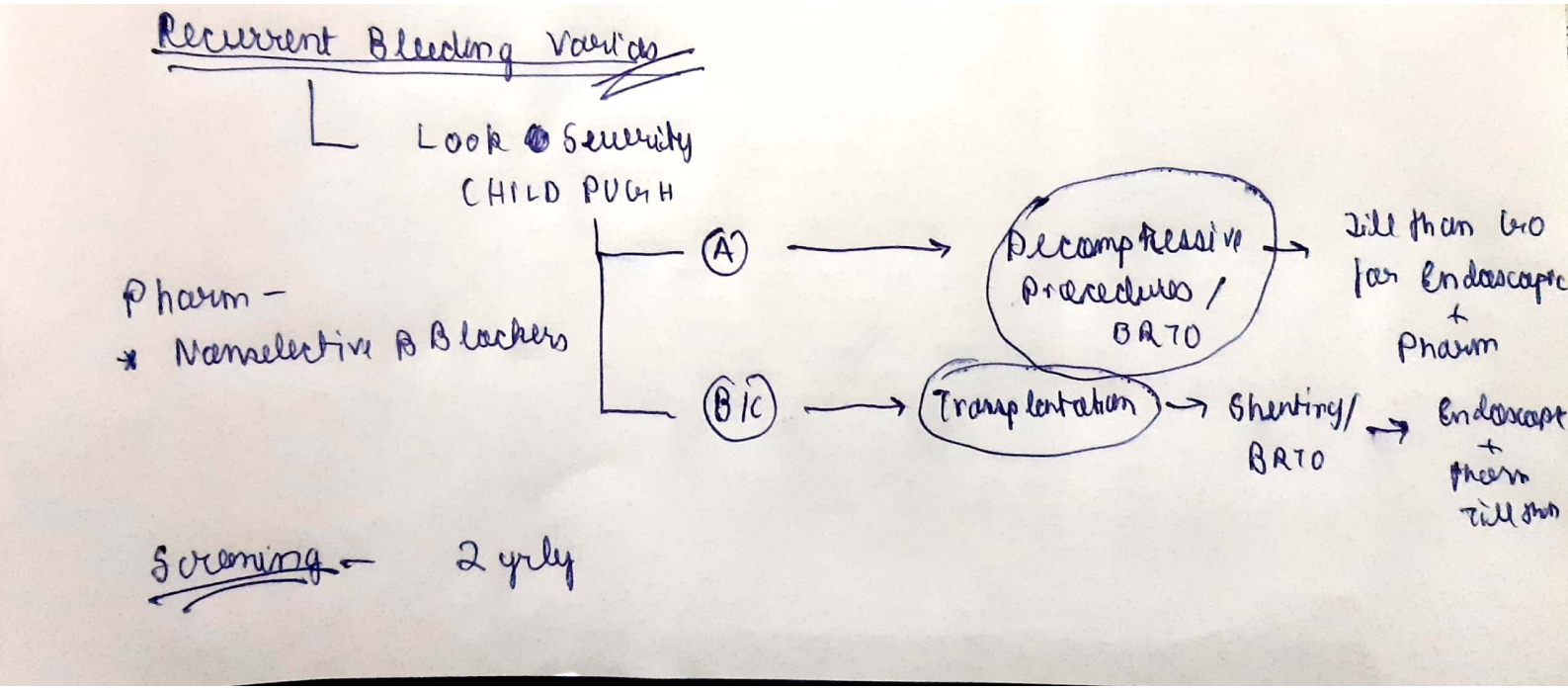
- Pharmacological –
  - * Vasoactives – IV – Terlipressin, desmopressin, Vasopressin, Octreotide
  - * PPI – Gro for Intensive Rx
  - * Lactulose
- Non Pharm –
  - * Initial – Balloon tamponade
  - * Endoscopic – band ligation (2-4wkly)
    - Sclerotherapy (Not used now adays)
    - Cyanoacrylate glue (Gastric)
  - * Decompressive procedures
    - TIPSS – Transjugular Intrahepatic Porto Systemic Shunts
    - Surgical Shunting
  - * More specific for Gastric Varices
    - BRTO – Balloon occluded Retrograde Transvenous obliteration
  - * Surgery – Oesophageal Transection ; Liver Transplantation

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Balloon-occluded retrograde transvenous obliteration (BRTO)



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Screening – 2 yrly

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Qns

Q. M/c cause of hospitalization due to UGIB.

Q. Endoscopic Rx for Mallory Weiss tears only in _____.

Q. M/c cause for Erosive Gastropathy?

Q. 2 Most specific symptoms of UGIB.

Q. Name of the score used for assessment of UGIB?

Q. Score less than which discharge & OPD Rx is possible?

Q. Hb criteria for Blood Transfusion?

Q. Time period for Endoscopy?

Q. Aspirin in established CVD/stroke needs to be _____?

Q. Interventional radiological procedure specific for Gastric varices?

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Ans:

→ Peptic Ulcer
→ Active Bleeding
→ NSAIDs
→ Haematemesis & Melena
→ Glasgow Blatchford Score
→ ≤ 2
→ < 7 g/dl (Harrison)
→ 24 hrs, 12 hrs if suspected of varices
→ Started ASAP as bleeding stops
→ BRTO

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